INFORMATION TO KEEP WITH YOU

# My complete treatment handbook

## Bevacizumab + weekly paclitaxel

Understand your treatment. Know what to expect. Keep your plan to hand.

**Displayed calendar**
**28 days**
See the active treatment phase

**Treatment stages**
**1**
Explained in the treatment reference

**Regimen source**
**NCCP 00738**
Version 3

Bevacizumab 10 mg/kg is given on Days 1 and 15, with paclitaxel 80 mg/m² on Days 1, 8 and 15 of each 28-day cycle. Continue according to benefit, toxicity and the treating team's plan.

## Treatment stages

01 **Combination treatment**
28-day reference

Your team confirms the active stage and dates. The full treatment guide explains each step.

## Inside your handbook


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**My name** _____

**Hospital / oncology team** _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

# Understanding your treatment

Bevacizumab + weekly paclitaxel · National Cancer Control Programme (NCCP) 00738

## When to get help

**Emergency: call 112 / 999**

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

**Contact your oncology service immediately**

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Seek urgent advice for major bleeding, severe headache or visual change, chest pain, breathlessness, one-sided weakness, severe abdominal pain, fever or poor wound healing.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

## Your medicines and what they do

Control advanced cancer and symptoms while balancing side effects and treatment burden.

ANTI-VEGF TARGETED ANTIBODY

### Bevacizumab

Avastin or a prescribed biosimilar



**Blocks a blood-vessel growth signal**

Bevacizumab binds VEGF, a signal that helps blood vessels grow. Blocking it can restrict the blood supply supporting a tumour. It also affects normal blood vessels and healing, which is why blood pressure, urine and planned procedures matter.

VEGF signal → Bevacizumab binds VEGF → New-vessel signalling is reduced

**What to watch for**

Report bleeding, a wound that is not healing, severe abdominal pain, new swelling, chest symptoms or a severe headache. Tell the team before surgery, dental procedures or port insertion.

TAXANE CHEMOTHERAPY

### Paclitaxel

Taxol (conventional paclitaxel)



**Locks the cell division machinery**

Cells use tiny structures called microtubules to separate their chromosomes when dividing. Paclitaxel stabilises these structures so they cannot rearrange normally, interrupting cell division. It can also affect nerves and other healthy tissues.

Microtubules form → Structures remain locked → Division is interrupted

**What to watch for**

Report tingling or numbness before the next weekly dose, especially if it affects walking or hand use. Tell the nurse immediately about flushing, chest or throat tightness, itching, dizziness or breathing changes during infusion.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

## Why this treatment?

Bevacizumab blocks VEGF-driven blood-vessel signalling while paclitaxel disrupts cell division. Treatment selection considers the first-line setting, cancer biology, alternatives and the balance between disease control and vascular, bleeding and nerve risks.

**Your treatment setting**

First-line metastatic breast cancer · source-defined eligibility

**Your exact schedule**

Bevacizumab 10 mg/kg is given on Days 1 and 15, with paclitaxel 80 mg/m² on Days 1, 8 and 15 of each 28-day cycle. Continue according to benefit, toxicity and the treating team’s plan.

**Alternatives**

Discuss biomarker-directed, endocrine, chemotherapy and supportive-care options appropriate to this setting, including the option not to proceed.

## Why this treatment may be chosen

Discuss how this evidence and the available alternatives apply to your own situation.

SACTCHECK · CHAPTER 02

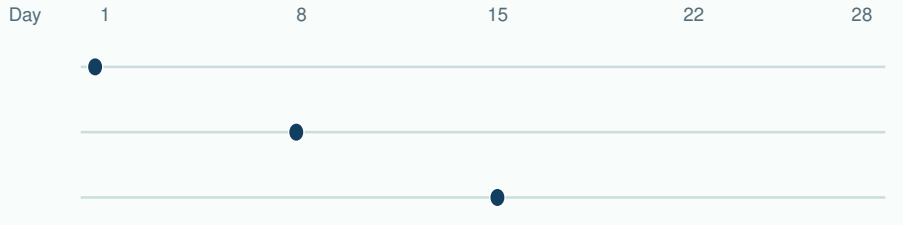
# Your schedule, tests and appointments

Bevacizumab + weekly paclitaxel · NCCP 00738

## Treatment and review

**Bevacizumab + weekly paclitaxel · 28-day reference**

28-day view




This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

28-DAY REFERENCE

### Combination treatment

**28-day interval**

Bevacizumab Days 1/15; paclitaxel Days 1/8/15. Day 8 has chemotherapy only.

Your team confirms the active medicines, treatment days, dose changes and next review.

BASELINE → TREATMENT → FOLLOW-UP

## When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm completion.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00738 v3 · official regimen · NICE NG101 · bone health and follow-up · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

## Bevacizumab + weekly paclitaxel: treatment and checks

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | First treatment | During treatment | Change / completion | Follow-up |
|---|---|---|---|---|---|
| **Treatment plan** NCCP 00738 | Confirm metastatic setting | D1 / D15 both medicines | D8 paclitaxel only | Repeat q28 days until progression / intolerance | Next treatment plan |

| Care / check | Before starting | First treatment | During treatment | Change / completion | Follow-up |
|---|---|---|---|---|---|
| **Blood / organ function** NCCP 00738 | full blood count (FBC), renal, liver | Cycle-start results | FBC including Day 8; confirm later doses | Review held / resumed medicines | Individual follow-up |
| **blood pressure (BP) / urine / wounds** NCCP 00738 | BP, urine protein, procedures | BP before bevacizumab | BP each bev dose; urine cycle plan | Surgery and healing review | Report bleeding / wound problems |
| **Cancer review** METASTATIC · MARKERS | Confirm setting | Agree review plan | Response / symptom review | Next treatment / completion | Agreed surveillance |

## What each check is for

### 1 Before treatment

Before the first dose

First-line metastatic breast cancer · source-defined eligibility. Baseline FBC, kidney/liver tests and urine protein. Review blood counts, renal/liver function and urine protein before cycles; Day 8 FBC is explicitly required. Check current counts for each paclitaxel decision.

**Ask / act:** Confirm eligibility, test results and medicine list.

### 2 Other checks

At the specified review points

Measure blood pressure before each bevacizumab dose and follow the agreed urine-protein plan. Review bleeding, clotting, wounds and planned procedures. Cardiac imaging is considered for prior anthracycline/chest radiotherapy or clinical concern; warfarin requires additional international normalised ratio (blood-clotting test) (INR) monitoring.

**Ask / act:** Confirm who arranges and reviews each check.

### 3 Support and practical plan

At each treatment or phase change

Separate the first paclitaxel premedication from later weekly doses; review blood pressure, urine protein, wounds and surgery timing for bevacizumab.

**Ask / act:** Choose the matching support plan; confirm active medicines.

### 4 Response and follow-up

Agreed review points

Agree the scan type and review interval for the known disease sites. Review response, symptoms and toxicity together; new or worsening symptoms may need an earlier assessment. Keep the agreed follow-up and report persistent or new symptoms between visits.

**Ask / act:** Review symptoms, results and patient priorities.

**At the next review, ask**

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

## Your treatment and access plan

STEP 1

### Before treatment

Baseline FBC, kidney/liver tests and urine protein. Review blood counts, renal/liver function and urine protein before cycles; Day 8 FBC is explicitly required. Check current counts for each paclitaxel decision.

STEP 2

### Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. A port is not automatically required; insertion and removal are separate procedures with individual instructions.

STEP 3

### On the treatment day

Confirm the medicines due that day, any premedicines and the reviewed blood results. Tell staff immediately about pain, burning, rash or breathlessness.

STEP 4

### After the visit

No routine take-home infusion pump is required for this regimen. Keep line-care, urgent-contact and next-review instructions.

#### Visit length

Bevacizumab: 90 minutes initially, then 60 and 30 minutes if tolerated; paclitaxel: about one hour. Full visit time is longer.

#### Line care

A PICC or port has its own care plan. Report fever, redness, swelling, pain, leakage or line damage; do not remove or adjust it yourself.

#### Treatment changes

A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

### Support for this visit

Separate the first paclitaxel premedication from later weekly doses; review blood pressure, urine protein, wounds and surgery timing for bevacizumab.

## Checks across treatment

Your team arranges tests and reviews the results before treatment decisions.



### Blood and organ checks

### Other treatment-specific checks

Measure blood pressure before each bevacizumab dose and follow the agreed urine-protein plan. Review bleeding, clotting, wounds and planned procedures. Cardiac imaging is considered for prior anthracycline/chest radiotherapy or clinical concern; warfarin requires additional INR monitoring.



### Cancer review and follow-up

Agree the scan type and review interval for the known disease sites. Review response, symptoms and toxicity together; new or worsening symptoms may need an earlier assessment.

### Bone health

Bone-health assessment follows endocrine therapy, ovarian suppression, menopause and other fracture risks. It has its own monitoring plan.

## The wider treatment plan

Review response, toxicity and priorities to decide whether to continue, change or stop. Supportive and palliative care can accompany anticancer treatment.



### Medicines review

Check new prescriptions, over-the-counter medicines and supplements with the oncology pharmacist before taking them.

### After treatment

Keep the agreed follow-up and report persistent or new symptoms between visits.

# Symptoms and supportive care

Bevacizumab + weekly paclitaxel · NCCP 00738

## Bevacizumab + weekly paclitaxel · 28-day reference

### Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

| Treatment or check | Timing in this view |
|---|---|
| **Treatment visit · Day 1** |  **Day 1** Bevacizumab + paclitaxel |
| **Treatment visit · Day 8** |  **Day 8** Paclitaxel only |
| **Treatment visit · Day 15** |  **Day 15** Bevacizumab + paclitaxel |
| **Blood / infection** |  **Days 1–28** Fever or sudden illness: seek advice |
| **Numbness / tingling** |  **Days 1–28** Report changes before the next dose |
| **Fatigue** |  **Days 1–28** May build across treatment |
| **Bleeding / blood pressure** |  **Days 1–28** Report bleeding, headache or new swelling |

**Timing varies:** Symptoms can occur on any day, including days without treatment. The bands are orientation, not predicted symptom-free periods.

**Treatment-day changes:** A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

**Why these bands stay broad:** These broad symptom bands are educational possibilities, not measured risk curves. Infusion days are the planned schedule; delays require a new clinical plan. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00738 v3 · official regimen ↗

**There are no guaranteed "safe days".** Fever, shivers, sudden illness, severe breathing changes or chest pain need prompt assessment on any day.

### Between-treatment notes

### Day 1–28: Reference cycle

Bevacizumab 10 mg/kg is given on Days 1 and 15, with paclitaxel 80 mg/m² on Days 1, 8 and 15 of each 28-day cycle. Continue according to benefit, toxicity and the treating team's plan. Follow your written appointment and medicine plan. A calendar does not authorise a dose.

## Support around the medicines

1 

**Blood pressure and urine**

Attend the required checks even if you feel well. Report severe headache, visual change or swelling.

2 **Operations and dental work**

Tell oncology about planned procedures; bevacizumab timing and wound healing need review.

3 **Paclitaxel premedicines**

The first dose and later doses have different plans. A previous reaction changes the assessment.

4 **Nerves and bleeding**

Report numbness, unusual bleeding or bruising before the next visit.

## Questions worth asking

### Who arranges the checks and explains the results?

Agree the due window, booking team, result reviewer and contact route if a test or appointment is missing. This page does not record completed checks.

**Try explaining the plan back:** Which medicines and doses are prescribed, when are they due, which checks are needed, and what would make you call the team today?

**Bevacizumab**

- High blood pressure / urine protein
- Bleeding or blood clots
- Delayed wound healing
- Rare bowel perforation / fistula
- Rare serious neurological effects
- Infusion reaction

**Paclitaxel**

- Low blood counts and infection
- Numbness / tingling, sometimes lasting
- Hair loss, muscle or joint pain
- Mouth, bowel, skin and nail effects

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**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## First paclitaxel dose · combination Day 1

**Active medicines:** Bevacizumab + Paclitaxel

**Emetogenic risk:** Low

| Medicine | Dose / route | When |
|---|---|---|
| Ondansetron | 8 mg orally once | Before each administered chemotherapy dose<br>Review QT risk, constipation, liver function and any steroid already prescribed. |
| Metoclopramide | 10 mg orally up to three times daily as needed | Rescue duration to be confirmed<br>Review contraindications, renal function and interactions. |

### Additional phase-specific support

- Before the first paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment.
- The first and subsequent infusion-reaction plans are different; do not automatically repeat the initial steroid schedule.
- Check blood pressure (BP) before each bevacizumab dose and reconcile urine-protein results, bleeding, thrombosis and wound healing.
- NCCP: withhold bevacizumab 28 days before major or 7 days before minor elective surgery. Do not restart for at least 28 days after major surgery and until the wound is fully healed; the team individualises this.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

## Day 8 · paclitaxel only

**Active medicines:** Paclitaxel

**Emetogenic risk:** Low

| Medicine | Dose / route | When |
|---|---|---|
| Ondansetron | 8 mg orally once | Before each administered chemotherapy dose<br>Review QT risk, constipation, liver function and any steroid already prescribed. |
| Metoclopramide | 10 mg orally up to three times daily as needed | Rescue duration to be confirmed<br>Review contraindications, renal function and interactions. |

### Additional phase-specific support

- From Day 8 onward, no routine dexamethasone is listed in the suggested table; chlorphenamine 10 mg intravenous (IV) 30 minutes before treatment continues.
- Famotidine 20 mg IV 30 minutes before may be omitted after tolerated treatment under consultant/local guidance. A previous reaction requires an individual rechallenge plan.
- Bevacizumab is not scheduled on Day 8; continue to report its adverse effects.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

## Later Days 1/15 · combination after tolerated paclitaxel

**Active medicines:** Bevacizumab + Paclitaxel

**Emetogenic risk:** Low

| Medicine | Dose / route | When |
|---|---|---|
| Ondansetron | 8 mg orally once | Before each administered chemotherapy dose<br>Review QT risk, constipation, liver function and any steroid already prescribed. |
| Metoclopramide | 10 mg orally up to three times daily as needed | Rescue duration to be confirmed<br>Review contraindications, renal function and interactions. |

### Additional phase-specific support

- From Day 8 onward, no routine dexamethasone is listed in the suggested table; chlorphenamine 10 mg intravenous (IV) 30 minutes before treatment continues.

- Famotidine 20 mg IV 30 minutes before may be omitted after tolerated treatment under consultant/local guidance. A previous reaction requires an individual rechallenge plan.
- Check blood pressure (BP) before each bevacizumab dose and reconcile urine-protein results, bleeding, thrombosis and wound healing.
- NCCP: withhold bevacizumab 28 days before major or 7 days before minor elective surgery. Do not restart for at least 28 days after major surgery and until the wound is fully healed; the team individualises this.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00738 · VERSION 3

# Bevacizumab + weekly paclitaxel

Bevacizumab 10 mg/kg is given on Days 1 and 15, with paclitaxel 80 mg/m² on Days 1, 8 and 15 of each 28-day cycle. Continue according to benefit, toxicity and the treating team's plan.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

## A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

**1 Blood pressure / vision**

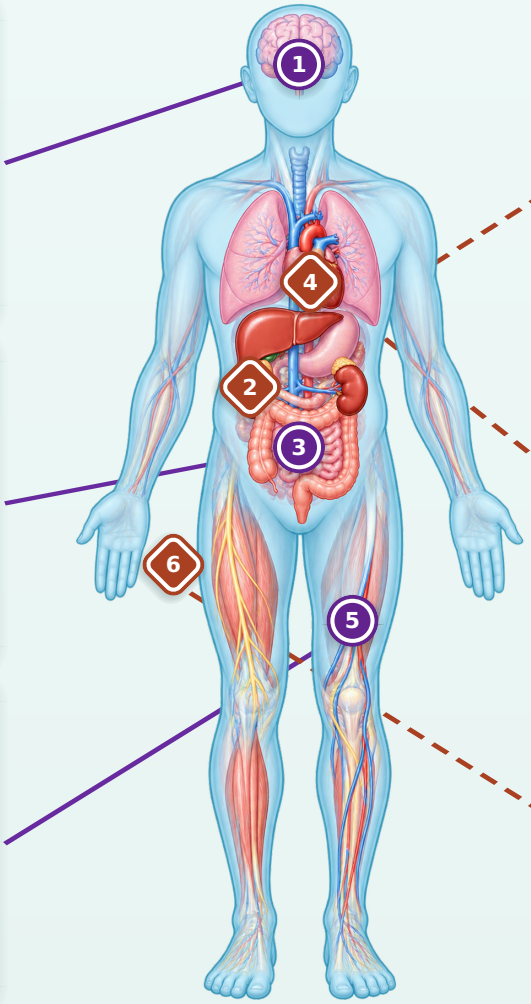
**Bevacizumab**

Severe headache or visual change needs urgent review.

**2 Urine / kidneys**

**Bevacizumab**

Protein checks can detect problems before symptoms; report swelling.




**3 Abdomen / wounds**

**Bevacizumab**

Severe abdominal pain or poor wound healing needs urgent advice.

**4 Bleeding / clots**

**Bevacizumab**

Report bleeding; sudden chest, breathing or stroke symptoms are emergencies.

**5 Blood / infection**

**Paclitaxel**

Fever, shivers or sudden illness: contact oncology.

**6 Hands / feet**

**Paclitaxel**

Report tingling or numbness before the next dose.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Selected medicine-specific effects; this is not a complete list. Report new or persistent symptoms even after treatment stops.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

SACTCHECK · CHAPTER 04

# Your record and contact card

Bevacizumab + weekly paclitaxel · National Cancer Control Programme (NCCP) 00738

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

**Treatment review · confirm active medicines** · copy this sheet for another cycle of the same phase.

## Bevacizumab · Anti-VEGF targeted antibody

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Bleeding / bruising** | □ None | □ New small bruise / brief bleed: report | □ Repeated bleeding: call today | □ Bleeding that will not stop: urgent help | □ Heavy bleeding / collapse: 112/999 |
| **Headache** | □ None | □ Mild and short-lived: record | □ New / persistent headache: call | □ Severe headache / visual change: urgent help | □ Sudden worst headache / weakness: 112/999 |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

## Paclitaxel · Taxane chemotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Numbness / tingling** | □ None | □ Mild change; usual tasks unaffected | □ Limits buttons, writing or walking: call | □ Washing, dressing or walking difficult: urgent review | □ Sudden major weakness / paralysis: emergency help |
| **Muscle / joint pain** | □ None | □ Mild aches; normal activity | □ Pain limits tasks / sleep: call | □ Pain limits self-care: urgent review | □ Sudden severe deterioration: emergency help |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |
| **Mouth soreness / ulcers** | □ None | □ Mild soreness; eating normally | □ Painful; soft food needed: call | □ Cannot eat / drink enough: urgent review | □ Major breathing difficulty: 112/999 |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/ day or at night: call | □ 7+ extra stools/ day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Nausea** | □ None | □ Queasy but eating and drinking | □ Intake reduced: contact the team | □ Unable to maintain intake: urgent review | □ Collapse / severe dehydration: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

# Carry your treatment details

Print, cut out and fold. Follow the contact plan from your own hospital.

SACTCHECK · SACT TREATMENT

## Bevacizumab + weekly paclitaxel

NCCP 00738 v3 · 28-day cycle

Bevacizumab 10 mg/kg is given on Days 1 and 15, with paclitaxel 80 mg/m² on Days 1, 8 and 15 of each 28-day cycle. Continue according to benefit, toxicity and the treating team’s plan.

| 1 | 2 | 3 | 4 | 5 | 6 |
|---|---|---|---|---|---|

**Your contact plan**

Oncology team: Insert numbers here
Out of hours: Insert numbers here.

Evenings / overnight: instructed out-of-hours pathway or nearest ED.
**Emergency: 112 / 999.**

Other hospital / own contact: _____

QR: https://sactcheck.com/patient/00738/ · no patient information.

Draft for review · SACTCheck v0.84.19

SACTCHECK · CHAPTER 05

# Sources and further information

Bevacizumab + weekly paclitaxel · National Cancer Control Programme (NCCP) 00738

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.


1. **National Cancer Control Programme (NCCP) 00738 v3 · official regimen**
   https://healthservice.hse.ie/documents/6671/738.pdf
2. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
3. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
4. **ESMO metastatic breast cancer guideline · 2026**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
5. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
6. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
7. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
8. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
9. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
10. **NOGG · section 7, aromatase inhibitors and fracture risk**
    https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
11. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf